Northbridge General Hospital (fictional)

100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Discharge Summary

Department of Orthopaedic Surgery

PATIENT	UHID	AGE / SEX
Priya Nair	**MG25004**	**67 years / Female**
REPORT DATE	REFERENCE NO.	DEPARTMENT
10 Apr 2026	**DSC-67968778**	**Department of Orthopaedic Surgery**

ADMITTED	SURGERY	DISCHARGED
05 Apr 2026	**06 Apr 2026**	**10 Apr 2026**

Diagnosis and Procedure

- Severe osteoarthritis of the right knee
- Right total knee replacement (TKR), cemented, performed on 06 Apr 2026 under spinal anaesthesia with an adductor canal block

Past Medical History

- Hypertension, treated with losartan
- No history of diabetes mellitus or cardiac disease
- Appendicectomy 1985

Allergies

No known drug allergies.

Blood Results

Test	Result	Unit	Reference range	Flag
Haemoglobin, pre-operative	**11.4**	g/dL	12.0 - 15.5	L
Haemoglobin, post-operative day 1	**10.1**	g/dL	12.0 - 15.5	L

Hospital Course

The operation was uncomplicated with a tourniquet time of 74 minutes. She was mobilised with a physiotherapist on the first post-operative day and progressed from a walking frame to elbow crutches. No blood transfusion was required. Knee flexion at discharge was 70 degrees.

Operative Details

Item	Detail
Surgeon	Mr. David Okoye (fictional)
Implant	Cemented posterior-stabilised total knee prosthesis, size 3
Anaesthesia	Spinal anaesthesia with adductor canal block
Tourniquet time	74 minutes
Estimated blood loss	250 mL
Tranexamic acid	1 g intravenously at induction
Wound closure	Layered closure with skin staples
Antibiotic prophylaxis	Cefazolin 2 g intravenously at induction, two further doses over 24 hours

Wound Care

The wound is clean, dry and intact with a waterproof dressing in place. Please keep the dressing dry and do not apply creams or ointments to the scar. Skin staples are to be removed on post-operative day 14 (20 Apr 2026) at the wound clinic,

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provided the wound has healed and there is no discharge or spreading redness. If the wound is slow to heal, the staples remain in place and the review is repeated weekly.

Recovery by Day

Day	Date	Mobility	Pain score (0-10)
1	07 Apr 2026	Sat out of bed; stood with frame	4
2	08 Apr 2026	Walked 10 metres with frame and physiotherapist	3
3	09 Apr 2026	Walked on ward; practised stairs	3
4	10 Apr 2026	Independent with elbow crutches; discharged	2

Medications on Discharge

Medicine	Dose	Frequency	Duration	Purpose
Enoxaparin	40 mg subcutaneous	Once daily	14 days	Prevention of venous thromboembolism
Paracetamol	1 g	Four times daily (QDS)	Ongoing	Analgesia
Tramadol	50 mg	Twice daily (BD)	2 weeks	Breakthrough pain
Lactulose	15 mL	Once daily	While on tramadol	Constipation
Losartan	50 mg	Once daily	Ongoing	Hypertension

Follow-up

- Wound clinic on post-operative day 14 for staple removal.
- Orthopaedic clinic at 6 weeks with knee radiographs.
- Outpatient physiotherapy three times a week from discharge.
- Contact the ward for calf pain or swelling, fever or wound discharge.

Electronically signed: **Mr. David Okoye, Consultant Orthopaedic Surgeon** on 10 Apr 2026

*** End of report ***

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